

METRO HEALTH SYSTEMS

Urgent Care Clinical Note & Radiology

Patient Name: Vance, Eleanor

DOB: 05/14/1968 (Age 58)

MRN: PT-8839201

Date of Service: 06/05/2026

Provider: Dr. Aris Thorne, MD (Urgent Care)

Allergies: **Penicillin (Hives)**

CHIEF COMPLAINT & HPI

Patient presents with a productive cough (yellow/green sputum), subjective fever, chills, and mild dyspnea on exertion for the past 4 days. Denies chest pain, sick contacts, or recent travel.

EXAMINATION

Temp: 100.8 °F | BP: 132/82 mmHg | HR: 88 bpm | RR: 20 | SpO2: 95% on Room Air

Lungs: Rhonchi and fine crackles localized to the right lower lung field. Dullness to percussion at the right base. Left lung is clear.

CHEST RADIOGRAPH (AP AND LATERAL) (ACCESSION #: CXR-2026-8819)

Indication: Cough, fever, suspected pneumonia.

Findings: Patchy focal opacity in the right lower lobe. No pleural effusion or pneumothorax. Heart size is upper limit of normal. Mediastinal contours are unremarkable.

Impression: Right lower lobe patchy infiltrate, consistent with acute community-acquired bacterial pneumonia.

DIAGNOSIS & MEDICAL DECISION MAKING

1. Acute Community-Acquired Pneumonia (Right Lower Lobe).

ALLERGY ALERT: Penicillin Allergy (Hives). Must avoid Beta-lactam antibiotics.

OUTPATIENT PRESCRIPTIONS SENT

- Azithromycin (Z-Pak) 250 mg | Day 1: 500 mg (2 tabs), Days 2-5: 250 mg daily | Qty: 6 tabs
- Benzonatate 100 mg capsules | 1 capsule PO TID PRN for cough | Qty: 20

Advised patient to go to ER if symptoms worsen, high fever persists > 48 hours, or breathing becomes increasingly difficult.